

I: Yeah.

I: Okay, so then we can start with the actual questions.

I: So the first question is about the familiarity with AI.

I: So could you describe how familiar you are with AI technologies in general?

P: Not really familiar. It's like 3 months since i discovered ChatGPT and i started to use it. So I'm very new to this.

I: Um, so could you also give a score on a scale of one to 10 of like how familiar you think you are?

P: Um, two.

I: Okay.

I: Um, and has your organization introduced any AI tools or systems in like your day to day?

P: No, not yet, but we are discussing it.

P: But not yet, no.

I: But there has been something you're thinking of.

P: Yeah, it has been something we have discussed in meetings, but it's only a point of discussion.

P: It's not really something we use.

I: Okay, well then we can go to the next question.

I: So this is for the diagnostics and assessment use.

I: So what are your thoughts on AI being used in diagnostic or assessment processes?

P: What do you mean with diagnostics?

I: You, for example, have intake meetings, right?

I: You could use AI possibly to help in those kinds of meetings.

P: Yeah, in intake meetings.

P: And the question was?

I: So what are your thoughts on AI being used in diagnostics or assessment processes?

P: Yeah, so I think that would be very nice.

P: especially in intake sessions, because we talk about a lot, right?

P: And we don't really have time to process all this information and to kind of make a nice

P: nice report on it because that's not something we get money for via the health care, the insurances.

P: So that would be great.

P: And I also

P: I know a couple of general practitioners that use it.

And it also helps us a lot when they use it because they don't have a lot of time as well.

P: But the AI makes like a really good summary of their sessions.

P: So I would be all for it.

I: Yeah.

I: So the main purpose you think it could have is like summarizing the interview process, right?

P: Yeah, like summarizing, that would be great, yeah.

I: And have you considered using it for maybe preliminary screening?

I: So for example, even before you come to talk with a psychologist, you could also check or let them communicate with an AI to kind of screen what the problems are, right?

P: Yeah, that would be good as well, because we see with screening that a lot of people that don't really belong in our section, they belong in the more specialized one, get through.

P: So I don't know how, to be honest, because we do questionnaires, but we do that when they are already in. So if you have an idea that would be great.

I: um let me see yeah so summarizing patient information. I think we can move on to the next one um so this is mainly for monitoring and progress tracking so how do you view the role of ai in tracking patient progress over time

P: then how would I imagine?

I: Well, yeah, so what is your opinion on the possibilities, possible risks?

I: What do you think you are doing now for progress tracking?

I: Or if it's something that's not useful or...

P: It could be useful, but there's always a risk, right?

P: That's what we are discussing as well with privacy because we attach so much value to this privacy part.

P: But I think for progress tracking, it would be great.

P: We are doing questionnaires now.

P: So at the beginning and in the middle and at the end, we let clients fill out three questionnaires

P: But that's it, but I'm not sure what the possibilities are with AI progress tracking.

I: Yeah, I mean, they're obviously working on a lot of different tools, right?

I: I don't want to say too much, because I don't want to influence your responses, but the main goal of this study is to like understand the possible use cases for AI and of course if there are some gaps between what people working on AI think and what psychologists think

I: But then also there are always limitations in making these things, right?

P: Yeah.

I: Let's see.

I: Yeah, so what concerns, if any, would you have about AI tracking?

I: That's mostly like privacy, right?

P: Yeah, that's I think the biggest part and also that clients don't agree to it.

P: So you have a bit skewed data then when some clients agree and others don't.

I: How are you going to do this?

P: Yeah, you can do it like the old-fashioned way, but

I: yeah do you think clients would not be open to uh to like talk with an ai for monitoring purposes or like other purposes at all?

P: um yeah I think it uh depends right I think that the younger clients they they I think they may be open to it but I think older clients yeah they're a bit skeptical I think uh so they may not be open to it

I: yeah but have you talked with any patients about this or is this just your own idea

P: no on on yeah this is just my own uh idea yes yeah i have not talked with

P: any patients about this now.

I: Okay, then we can move on to the next question.

I: So this is for administrative and practical support.

I: So how do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Also reminders for clients, for instance?

I: Yeah, you can do reminders for psychologists or reminders for clients.

P: Sounds good.

P: Yeah, because we also are discussing this, right?

P: Because a lot of, well, at least in this period of the year, a lot of clients, they...

P: forget their appointment or they just call in sick

I: is that specific for this time of the year

P: yeah yeah it's also it's kind of we had like last week was like blue Monday right so yeah it's this point of the year is yeah they have like a new year's resolutions and stuff like that and then

P: After a month, they slip up and motivation goes down the drain.

So, yeah, that's kind of for this part of the year.

P: But when they call in sick, it's like a gray area.

P: So we lose money.

P: When you have more kind of... ways to keep a client involved throughout the week or kind of give them reminders your appointment is almost there i think it would be very good for motivation you know

I: because the um like your planning and stuff that's probably done by some someone else or are you doing the the planning of actual meetings with your clients

P: Yeah, we have like a office manager.

P: So she is, uh, kind of scheduling the meetings, but I'm doing it as well, uh, with clients, uh, in the, in the room.

P: Um, so I don't think AI would be any, uh, like added value there, but I can be wrong Right.

P: I think that goes pretty okay and there's always like a important part of like treatments ending early or prolonging you're prolonging the treatment so that's then you can get gaps in the schedule right in the timetable

P: complex to always have like meetings right after each other or there's no gaps because it's changing every day with people being sick or treatment ending early or just people are not available on this day for intake conversation stuff like this so

P: I think there could be some added value there, but I'm not sure how.

I: Yeah, because the actual appointments are very dynamic, right?

I: You have people just make an appointment one day or a week before and then canceling two days before.

P: Yeah, could be.

P: We have like a standard of 12 sessions.

P: So it's pretty short.

P: But yeah, it's always possible that someone cancels on the day itself.

P: And when someone is sick, yeah, you can't really say, okay, we're going to send you a bill because you're sick.

P: It's kind of a gray area.

P: We always tell patients to call 48 hours before.

P: But yeah, that's not always...

P: possible you can really uh see in the future if you're gonna be sick

I: Yeah and the 12 sessions is that for all uh cases

P: Yeah it's kind of uh because we kind of introduce in the intake conversation that we are have like uh it's it's called the yeah in dutch it's called "Kortdurend behandelen" —short term treatment—

P: I'm not sure how you say this in English.

P: So yeah, that's kind of our standard and it can be very effective because it also helps with motivation.

P: But yeah, I do a lot.

P: Like the last four sessions, I take one week in between, for example, when some client is doing better

P: So they can practice more.

P: But then you have gaps, right?

I: Yeah.

I: So every other week, right?

P: Yeah, every other week.

P: Yeah.

P: I don't know if that's an answer for your question.

I: No, I mean, just any, any answer is the correct answer, right?

P: Yeah.

I: Um, yeah.

I: So then, uh, you normally do 12 sessions and then I'm guessing that maybe in the beginning they need more help.

I: So the, the meetings are like more close to each other.

I: And then at the end, they kind of, uh, start having like some gaps, right.

I: And that leaves some opportunity maybe to schedule better or whatever.

I: Right.

P: Yeah, true.

P: And I think there,

P: AI can come in handy to kind of do something with this, right?

I: Because it's probably like a lot of managing on your part and then also on the office manager, right?

I: Like, yeah, it might be that there is something helpful there.

P: Yeah.

I: Do you have any concerns about accuracy and liability in that regard?

P: Yeah.

P: Um, yeah, of course it's, uh, you're, you're, you're not for, I think at least in the beginning, I'm not sure how accurate it's going to be, but you have this feeling of, okay, I'm not really in control.

P: There's some program doing this for me and I, I have put, I have,

P: right, to pick out the right things.

P: So I think, yeah, that would be kind of a bump you have to get over.

P: Yeah, just kind of see, like an experiment, how it's going to go.

P: Yeah, of course, yeah.

I: yes okay um we can move on to the next part so this is for psychoeducation and between session tools um what are your thoughts on using ai for psychoeducation or therapeutic exercises between sessions

P: Um.

I: like are you doing any anything in between the sessions already or?

I: I mean like possible homework or other tools that they might use between therapy sessions

P: Yeah, I always do this.

P: It's very important because

P: I always say the biggest change is out of the sessions.

P: I can say whatever I want to you, like a hundred different things, but when you don't do anything between the sessions, then it's not going to work.

P: Maybe that's a bit strict, but there's definitely some truth in that.

P: So yeah, of course.

P: it could be it could help as as long as it doesn't contradict what i'm saying right because then you have yeah you you have to know what the ai is going to say because otherwise we may be saying uh similar things uh or i mean yeah so yeah i think it could be could be good but you have like uh protocols right you you know this

I: uh yeah yeah i mean there's like different types of ai of course right like and also very different types of applications you could use it for um yeah but this is a possible limitation.

P: I think it can be uh confusing yeah that's and also it can be a lot of time investment because um yeah

P: I see a lot of clients with burnout, right? Or depression. And then you kind of want to be a bit wary of the amount of work you give these clients.

P: And when they also have to talk with this AI bot, then it could be a lot.

P: Yeah, there are a lot of clients that think a lot, right?

P: And then mostly negative thoughts.

P: That's why they come to a psychologist.

P: So when they're starting to kind of ruminate with this AI bot, that could be a bit troubling, right?

P: You can also spend like four hours talking with an AI bot, like asking all the questions you have.

P: So yeah, I think that, yeah, it has to be,

P: like carefully made for those two type of clients.

I: Yeah.

I: Like I have two questions.

I: So what are, what are the main kind of homeworks you give your clients?

I: Like what type of homework do you give them?

P: The homework I mostly give is like challenging.

P: uh negative thoughts and kind of uh putting like a more helping or more positive thought uh besides this negative thought to kind of give a more nuanced uh nuanced thought in the end yeah um so you have a lot of forms uh for that so i give that uh that's the most

I: so that's mostly people with like negative negative thoughts they should like recognize okay i have this negative thought now let's try and reframe this or come up with some other explanation or some other feeling yeah that could replace or like uh kind of uh fight this general negative idea right

P: yeah it's not really replacing but it's okay to have this this negative thought that's yeah yeah you can kind of nuance it by

I: so it's not only negative

P: yeah yeah yeah true yeah so that's that's cognitive behavioral therapy right yeah uh so i do i do that uh a lot and also i do some things with acceptance and commitment therapy i don't know if you know this uh you also have homework uh with uh uh looking into your values and having like uh aimed action towards your values stuff like this.

I: okay then we can move onto the next part oh actually i i do have a question so do you think um so what you're saying right like there there is a possibility of the AI contradicting your views um so what would you think would be the right course of action in that case like um because

I: for you to know that the AI is contradicting you, either your client has to tell you that or you might be forced to monitor what the AI is telling the client, right?

I: And that would, of course, also create more work for you.

I: And also maybe some privacy problems.

I: I don't know.

I: So there's definitely some risks and limitations there.

P: Yeah, the biggest issue is time in psychology.

P: So the work pressure is pretty high because of insurances, right?

P: We can't kind of send a bill for everything to the insurance company.

P: So yeah, that will be like the

P: the biggest thing, so I can't really monitor everything, right?

I: Yeah, AI can be... Yeah, so actually, that is true.

I: So you're saying that you... Because I was thinking maybe you should be monitoring this, but it could also be a problem possibly with insurance that monitoring does not fall under insurance, right?

P: No, it doesn't.

I: Yeah, so then it would just mean that

I: either insurance has to change, which is probably a very hard thing, right?

I: Insurance is a very, very big problem.

P: That's very optimistic.

I: Yeah.

I: Then there has to be a lot of changes in mental health care, probably.

P: Yeah.

I: But it is possible, of course.

I: We don't know.

P: Yeah.

I: Yeah.

I: But

I: yeah like the the monitoring of ai like in its early stages most likely will be needed and it's very hard to um get coverage with insurance probably right

P: um yeah i think in the basic uh health care it is uh but in specialized uh i think there's more room for it because there's also more monitoring in general yeah and you can also kind of explain it as uh diagnostics um so i think it is possible more, so for example

P: by sending them some overview or something, because that's it, right?

P: You need to have direct client contact in order to get it insured.

P: But in Specialized, there's more room for it because you have a bigger diagnostic package, let's say it like that.

P: So it's not like it's not possible.

P: You have to think of like a smart way to kind of explain it to the insurance companies, right?

P: So it's definitely possible.

P: And also in the basic healthcare, but I think that you have to kind of think of...

P: Like a clever way.

I: Clever ways of reframing the monitoring to be, yeah.

P: Yeah.

I: Okay, let's see.

I: Let's see the time.

I: Okay, so now we'll just go into risks and limitations.

I: So what risks or concerns do you have about using AI in clinical work?

I: We've kind of already mentioned some, but if there's any other or anything you want to elaborate on.

P: Let's think.

P: I think we spoke about a lot, right?

Yeah.

P: I think that the biggest thing is that you have to be very careful of kind of mentioning names and stuff like this.

P: Because there's always a possibility of the information being leaked.

P: And if that happens one time, then you're screwed as a practice, right?

P: So I think that it's a big issue why many practices don't really use it.

P: Because it's, yeah, I'm not sure if it's that unsure, right?

But I think a lot of psychologists have this feeling.

P: It can go wrong one time and then they are screwed.

I: Yeah.

I: Okay.

I: So, so during this, uh, conversation, like the, the main risks that you mentioned and correct me if I'm wrong is just the privacy, right.

I: And, uh, the conflict of, uh, possible ideas that the AI might have, and you might have, or have agreed on with your client.

I: Um, and then there was one more, I think.

P: Yeah.

P: Yeah.

P: also the summarizing if it is kind of correct yeah yeah for the intake sessions um yeah i think that's it and also the the monitoring right?

I: Yeah, let's see.

I: So this is also something we kind of talked about.

I: So the client perspective and therapeutic alliance.

I: How do you think clients respond to AI being used in their treatment?

I: We kind of already mentioned like some people might not be willing to participate with AI being used or some people are.

I: Do you have any more thoughts on that?

P: Yeah, it differs, right, for each client.

P: I'm trying to think of my own perspective.

P: If a psychologist asks me if they can use AI, and I would say, yeah, I think I will be fine with it.

P: I think I'm also a bit hesitant when I'm speaking in conversations what I'm what I'm saying or mentioning names I think I think that maybe like holding back information because you maybe because that's also can be because I'm pretty new to to AI and stuff like this but that's that's my perspective I think there are people like

P: you for instance that know way more about AI that may think yeah it's absolutely fine because people are pretty open and also because we always ask if we can contact the general practitioner

P: 99 percent of the people say yes so they are not really bothered by possibly us sharing some information with the general practitioner as long as it can help them yeah so I think people are also pretty open yeah when you explain it in a correct way

I: Yeah, I think that's very true.

I: I think you need to... For some people, they might be hesitant in allowing it, but I think if you have a clear way of explaining what you're actually using it for...

I: if you say oh can we use your data or can we record the audio or whatever and we'll just feed it into an ai and they don't know really what what's going on then i think they might be more hesitant than saying oh we will just record audio and like delete it afterwards but we will use this recording to just make a summary so it's like easier to um keep track of your progress or something

I: If that's the explanation that's probably more acceptable for people, than we'll just use it for AI, right?

I: So I think definitely there is some factor where you need to explain why you're doing things and how you're doing things.

P: Yeah, because it's not really a normal thing.

P: at this point.

P: It's still new, so I think you have to explain it.

I: Could you also give a score of 1 to 10, where 1 that clients don't respond well and 10 being they respond well

P: Maybe a 6, but it depends on the group and how you explain it.

I: Yeah, let me see.

I: Okay, I think that's fine.

I: And also with time, we should just...

I: Okay, last question.

I: So this is about the professional identity, autonomy and future conditions.

I: Which aspects of your work should remain strictly human?

I: And what would you need to feel confident about using AI safely in practice?

P: Well, I of course think like...

P: client psychologist contact has to stay human.

P: I don't think it has to stay a fully, fully human.

P: I think you can do some things online as well, which can add to your, to your treatment.

But I think like making like summaries of intake sessions and

P: stuff like this, uh, it would be very nice if I don't have to do this because it's not really, I have to find like time in between sessions to do this because it's not something that is, uh, uh, insured or how do I say this?

P: Uh, we can really, uh, uh, make money with, with this.

P: Uh, so that, that would be a big, a big win.

P: Mm-hmm.

I: Yeah, so honestly, you just think the main conversation should still stay human, right?

P: Yeah.

I: Have you had any online meetings with your clients as well or not?

P: Yeah, I have.

P: Yeah, yeah.

I: and and do you see a clear difference between those kinds of sessions like with the in-person session and the online session

P: yeah i see i see a pretty big difference because uh yeah just like like now i don't see a lot of uh non-verbal uh communication right so for me

P: And that's just preference because I know practices that do like full treatment online.

P: And I know, uh, uh, science is kind of also backing that for like, not that severe, uh, uh, conditions.

P: Uh, the result is equal for online and, uh, in person.

P: Um, but for me, and that's, that's, uh, that's my preference.

P: I like to just do it like, uh,

P: in person and not online because I, I can, um, yeah, I don't, I see, I don't see everything.

P: Right.

I: So you think online there is some chance for detachment

I: Right?

I: Like you, you just don't feel the, the emotions in the room.

P: Yeah, that's true.

P: And also because, um, you're looking not really at me and I, you know,

P: I don't really look at you, right?

P: So that's already a bit hard to kind of get this like connection you would get in person

P: but that's just preference yeah yeah

and there might be some people that have a preference for doing it online and some people that might want to do it in person right

yeah yeah you have like I practice in the Netherlands I know some people there as well and they also do treatment

P: online and they also combine it with treatment in person.

P: So they have like an online platform and they kind of do like exercises with the clients.

P: And then these clients also have sessions in person with the more senior psychologists.

P: So they make this combination of online

P: And in person, but they also do like full treatments online if it's not very severe.

I: Yeah.

P: Yeah.

I: And yeah, so you also think that the more severe cases might benefit more from in person than or like maybe other way around.

I: So less severe cases would be maybe easier to do online than the more severe cases, right?

P: Yeah, yeah, yeah, yeah.

P: Because there's also benefits for online, right?

P: You are in your own safe space.

You don't have to travel, stuff like this.

And I think from science, I don't know if it's outdated, but...

they actually found that more severe cases, they benefit more from in-person than online.

P: But if you have like mild depression or light depression, for example, then the outcome is equal.

Yeah.

P: Yeah, it's interesting.

I: Yeah, for sure.

P: You can look it up.

I: Yeah.

I: um okay so that's kind of it um is there anything you want to add or

P: um no i think we uh talked about a lot

I: yeah yeah um okay let me just stop the recording